

IT'S NOT YOUR FAULT

A Cardinal's Toolkit Guide

For Families Caring for an Aging Parent

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CHAPTER 1

Something Has Changed

In 2015, my phone rang. It was Dad.

That alone told me something was wrong. I called him. He didn't call me.

His voice was cracking.

"Rob, I have cancer, and I don't know if I'm gonna make it."

The tumor had taken half his esophagus and half his stomach. There had been no warning. No symptoms we'd missed. No appointment that raised a flag. Just a phone call on an ordinary day.

I sat down on the couch. I had to. My chest was tight and I needed to catch my breath before I could answer him.

Everything in me wanted to say, *It's going to be okay*. I didn't say it.

I said, "We'll do everything we can, Dad. And we'll respect your wishes on how that's done."

He decided against chemotherapy. That was his wish, so that was the plan.

I was at work when the call came that the surgery was scheduled. I let my boss know I'd be gone for the days that followed. I was going to be there for the recovery. That part was never a question.

At the University of Alabama at Birmingham, surgeons used a da Vinci robot to take out half of his esophagus and half of his stomach.

Then came the infection. He ended up in the ICU, hallucinating. The whole family came to town. We stood around his bed and we did not know if he would make it.

My dad is a fighter. He made it.

That was 2015. He lived ten more years. And everything that fills this book — every decision, every hard day, every conversation — started with one phone call I never saw

coming.

Yours may not be a phone call. It may be a fall. A missed bill. The same question asked three times in an hour. But there is always a moment when something changes. This chapter is for that moment.

And whatever you think you should have seen coming before your moment — you couldn't have. We couldn't either. It's not your fault. That's why those words are on the cover.

CHAPTER 2

Falls, Weakness, and Decline

In 2023, my wife and I were at home on the couch, watching a movie on Netflix. An ordinary night.

The phone rang. Dad had fallen and broken a couple of ribs.

Here is what you need to know about my father at that time: he was going to the gym on a regular basis. With a personal trainer. This was not a frail man shuffling toward a fall. This came out of nowhere.

At the hospital, they scanned his chest for the ribs.

The scan found a tumor.

Eight years earlier, I had taken the first call sitting on a couch. Now I was on another couch, taking another one. The same helpless feeling, arriving the same way — in the middle of an ordinary night.

Dad made his decision the way he always did: his call, his terms. This time he said yes to treatment. He started immunotherapy.

But hold on to this part. The fall didn't cause the problem. The fall *found* it. If he hadn't gone down, if those ribs hadn't broken, no one would have looked inside his chest.

A fall is never just a fall. Weakness is never just getting older. They are information. Read them early enough, and you get to make decisions instead of reacting. And nobody catches everything — I didn't, and this is my profession. But once you know what a fall is trying to tell you, the next one becomes information instead of just fear.

CHAPTER 3

Memory Loss and Dementia

My first experience with dementia was professional. Home health — other people's families, other people's parents. My first real one was Nana, my mom's mom.

She was in rehab. I walked into her room and she was lying on the bed.

"They're stealing my socks in here," she said.

And then: "They're having their way with me."

Stop right there. Hear those words from your own grandmother and everything in you reacts. Part of you wants to panic. Part of you wants to correct her — *Nana, nobody is stealing anything.*

I did neither. I asked.

"What do you mean, Nana?"

She told me more, and I kept going, one gentle question at a time. Peeling the onion.

"Do you mean they're working with you — because of the therapy?"

"Yeah," she said. "That's what they're doing."

The therapists. Strangers moving her body through exercises she didn't remember agreeing to, in a building she didn't remember arriving at. Her words were frightening. Her reality made sense — once I stopped standing outside of it and started asking about it.

I want to be careful here, because words like hers deserve care. When a person with dementia says something like that, you take it seriously. Every time. Ask the soft questions, the way I did. And look: for bruises nobody explains, for fear of one particular face, for a change with no story behind it. If the answers don't add up, call the administrator and the long-term-care ombudsman, and don't apologize for it. That day, every answer pointed to the therapy room. That is the only reason this story ends gently.

There were other changes. She told stories from many years ago as if they had happened that morning. And there was a crudeness that was never her — my grandmother was a proper woman her whole life. The disease took her memory. It took her manners too.

I sat with her. I didn't argue. And afterward I told my mom the thing I want to tell you: there is no point in arguing. Go with the flow, and try to understand what her world looks like from the inside. You will never win an argument with dementia. But you can still win the visit.

CHAPTER 5

The Family Meeting

Here is the truth about family meetings: the one that goes wrong is the one that never happens.

I know, because it happened in my family. And I do this for a living.

Through everything — the cancer in 2015, the tumor the fall found in 2023 — we never sat down, all of us, and talked about what would happen if Dad declined. His will. His house. His money. What he wanted, and how any of it would be paid for.

He had told my stepmom some things. He wanted to be buried; she knew a few of his wishes. But that was it — pieces of a plan, living in one person's memory. There was no meeting. There was no list. And none of us asked the money question. Not one of us. Not even me, and I do this for a living.

We didn't avoid it out of carelessness. We avoided it out of love — the kind that doesn't want to say the hard thing out loud, as if the conversation itself could bring the day closer. It doesn't. The conversation only decides whether you will be ready when the day comes on its own.

This chapter is the meeting my family never had. Have it.

CHAPTER 9

Memory Care

Let me tell you what I found behind the locked doors of a memory care unit.

On one unit, the staff had painted a mural on the wall: a bus and train station.

Every day, someone would head for the door. *"I gotta go home."* *"I'm not supposed to be here."* Those may be the most common words spoken in memory care, and some of the most heartbreaking. Everything in an untrained person wants to answer them with the truth — *you live here now* — and that answer only turns fear into a fight.

The staff never argued. They invited.

"Why don't you sit and wait for the train with me?"

And the resident would sit. On a real bench, at a painted station, waiting for a train. When the worry settled, the redirect came, gentle as anything: "Train must be running late. Let's go get something to eat while we wait."

By the time the meal was done, the urge to leave had passed the same way it arrived — as a fleeting thought.

Some people call that a trick. It isn't. It is meeting a person inside her reality instead of dragging her out of it. Nobody was corrected. Nobody was restrained. Nobody lost an argument about where home is. My Nana taught me you will never win an argument with dementia. That painted station is what winning looks like instead.

The other thing I watched work behind those doors was music. Play a song from a resident's younger years — the music of their formative decades — and someone deep into dementia would recognize it and sing the lyrics. Every word. The disease takes so much, but it takes the music last.

When you tour a unit, look at the walls.

CHAPTER 10

Skilled Nursing and Rehab

Marion and Frank are family friends. Frank fell and broke his hip, and he was admitted to the local hospital. His daughter stepped in as his primary caregiver — and the family called me with one question: *where should he go for rehab?*

I gave them my recommendation. That part took five minutes.

Here is what I carried into that call that they didn't — the thing every rehab team I ever worked alongside knew: a broken hip after seventy-five is one of the hardest recoveries in all of elder care. A significant number of people who go into rehab for a hip never make it back to the life they had before. Many will need a higher level of care when rehab ends.

I didn't lead with that. Because here is the other thing I know, just as true: recovery varies person to person, and nobody's outcome is written in advance. You give rehab everything you have before you ever assume the next level of care is coming.

So that's what I told them, and it's what I'm telling you. Hold both. Cheer the recovery like it's going to work, and quietly start learning what comes next in case it doesn't. That is not pessimism. That is showing up prepared.

Frank is fighting, and I check in on the family. But you may not have a friend in the industry to call. This chapter is that phone call.

CHAPTER 12

Hospice

Hospice is the word families fear most in this book. So I want to tell you how I came to love it.

I got sober on January 9, 2009, and part of staying sober is helping other people get sober. Some of the people I sponsored didn't make it. They were young — most of their life still owed to them — and sitting with their families afterward is where I learned what a robbed life looks like, and how to stay in a room full of pain without running.

A hospice patient's life is not a robbed life. Most have lived full ones — work, marriages, grandchildren around a table. And when they haven't, hospice holds those families closest of all. Hospice takes nothing from anyone. It gives. Comfort instead of pain. Somebody in the room instead of machines. When I tell a family that choosing hospice is a gift, I am not softening anything. I have seen the alternative, and it is not a longer life. It is a harder death.

Then came Christmas Day, 2025. My dad.

At the end, his comfort was real. He did not suffer his way out of this world. And I want to say something plainly, because I know how end-of-life comfort care gets read: the morphine does not bring the morning closer. It is started because the morning has come. Given right — the right dose, for the pain that is actually there — it takes nothing but the suffering. And that comfort wasn't luck, and it wasn't inside knowledge. It is what a hospice team brings into every home. Including yours.

Now when I sit with a family, I am not speaking from training. I have lived the same pain they are living — including the weight of the decisions that fall to the family at the end, when the patient no longer can make them.

Hospice is not giving up. It is the most active form of love available. It says: *I will not let you be in pain. I will not let you be alone in this. I am a phone call away every hour of every day and night, and I will keep showing up until the last day.*

CHAPTER 17

Prescription Assistance

I want to tell you about a patient who lived in a million-dollar home — because the family almost didn't fill out the paperwork.

Why would they? Look at the house. Nobody in a home like that qualifies for assistance. Everyone knows that.

Everyone is wrong. Prescription assistance from drug makers and charities is based on *income*, not net worth. Nobody counts the house. What counted was what came in each month — Social Security and investment income — and that number fell below the threshold. The patient qualified.

So here is the rule: **always fill out the paperwork.** Do not decide for the program whether you qualify. Do not let the house, the boat, or your pride answer a question the application never asks. Answer what it asks, send it in, and let them say no.

One warning before you run with that rule: some programs later in this book do ask about savings. Medicaid asks hardest — for that one, get a guide before you file. But never do a program's math for it.

Most families never apply. The money goes unclaimed, year after year, left on tables by people who assumed.

CHAPTER 19

The Important Documents Folder

Let me show you how a person ends up on a double dose of blood thinner.

The cardiologist prescribes it by its brand name. The primary care doctor — different office, different network, computer systems that do not talk to each other — prescribes the same drug as a generic. Two different names. Two different bottles. The same drug inside both.

The patient fills both prescriptions, because a doctor ordered each one. Takes both, because they're on the med list. And nobody catches it — because the two doctors have never spoken, and the only person who ever sees every bottle in the house is the family.

For painkillers and a handful of other controlled drugs, there is a partial safety net: state monitoring programs a pharmacist or doctor can check. For everything else, there are exactly two nets. The first is simple, and I am begging you to use it — fill every prescription at one pharmacy, so one computer sees every bottle. The second net is you.

That is what the Important Documents Folder is. The net, on paper.

One folder. One master list. Every family member involved in the care has it: the cards, the doctors, every diagnosis, and the medication list — updated the day anything changes, with which doctor prescribes each drug, and brand and generic names side by side. The full checklist is in this chapter.

You cannot count on doctors in different systems to communicate. That sentence should be printed on the front of the folder, because it is the reason the folder exists. You are the only one who sees the whole picture. This chapter puts the whole picture on paper.

It may be the least dramatic chapter in this book. It may also be the one that saves a life.

CHAPTER 20

Healthcare Power of Attorney

There is no such thing as too early for a healthcare power of attorney. There are, however, many kinds of too late.

A friend of mine learned this with their own mother. Her dementia had been progressing for a while — and when the family finally reached for the paperwork, the window had closed. A person has to be capable of understanding what they are signing, and she no longer was. She could not put her own choice on paper anymore. She could not name which of her children spoke for her, or make sure the hospital would listen to the right one without a fight.

Most states will let family step in eventually, even without the paperwork. What my friend lost was the eventually. Every decision came with delays, disagreements, and doors that a single signed page would have opened instantly. Everything was harder, slower, and colder than it ever needed to be.

Here is the part I need you to sit with: dementia does not announce the last day your parent can still legally sign. That day arrives quietly, looks like every other day, and passes.

And here is the other half, because you may be reading this with a diagnosis already in the family: a diagnosis is not the closed window. Many people in early dementia can still legally sign — the question is whether they understand what the document does, and an elder-law attorney can tell you in one visit. Go this week.

And if the window truly has closed in your family, keep reading. There is still a path — this chapter covers it — and missing a window nobody showed you is not the same as failing.

Get the healthcare power of attorney done now, and put it in the folder from Chapter 19.

CHAPTER 21

Living Will / Advance Directive

Picture the hallway outside a hospital room. On one side, a spouse. On the other, the grown children. One of them is saying *keep treating*. The other is saying *let them rest*.

I have seen this fight, and I want you to understand something about it: both sides love the same person. Both sides believe they are defending that person's wishes. And both sides are guessing — because nothing was ever written down.

Spouse versus children is the most common version, and it is the cruelest, because it sets the two halves of a family against each other at the exact moment they need each other most. A living will ends most of that fight before it starts. The question stops being *what do we think they'd want* — with everyone's grief and fear and history piled onto the answer — and becomes *what did they say*. On paper. In their words.

In Chapter 5, I told you my family never had the meeting. A living will is what the meeting writes down: CPR or not, machines or not, where the line is between fighting for life and holding on to it too hard.

And it has to be *findable*. A DNR that exists in a drawer nobody can locate does not exist — if the paper cannot be produced, the default is full intervention, including the things your person said they never wanted. Chapter 23 covers where these orders must physically live. For now: write it down, and make sure the right people can put their hands on it in under a minute.

CHAPTER 22

Financial Power of Attorney

Everything in a care crisis costs money, and here is what most families learn too late: when no one has the legal authority to move that money, everything stops.

The rent at the assisted living community. The insurance premium that keeps coverage alive. The bills that keep the lights on in a house nobody's living in right now. Without a financial power of attorney — or with the wrong person named — the family can't act, and a court and a judge step in. Guardianship proceedings, oversight, months of it. The court usually ends up appointing a family member anyway — after the delay, the cost, and the strangers added to your family's worst month. And while all of that grinds forward, everything waits. So does the person it's all for.

The urgency is identical to the healthcare POA in Chapter 20, and so is the window: it closes when capacity does, quietly, without notice.

So here is what I want you to do. Get all three documents done together — the living will, the healthcare power of attorney, the financial power of attorney. One effort, one afternoon if you can manage it. Do it with someone who knows your state's rules, because signing requirements vary — witnesses, notaries, and who is allowed to be which.

And while you're at that table: **do your own.**

You are reading this book because your parent's crisis landed on you unprepared. Someday, your crisis will land on someone you love. The three documents you are gathering for your mother are the same three your children will need from you. Do them at the same table, on the same day. It is love, in paperwork form.

CHAPTER 24

Advance Care Planning Conversations

Just last week, I sat with a family and helped them have the conversation — the one about what's coming, what he wants, what needs saying while there's still time to say it. I have that conversation with other families all the time. Don't leave anything left unsaid.

Now let me tell you what I got wrong with my own father.

I never forced the planning conversations. I knew better — it is literally my work — and I still let them slide, the way every family does. There was always going to be more time. So the decisions he should have made stayed unmade, and they came due the week of his funeral. Money questions, surfacing in the worst week to face them.

That is one kind of unsaid, and I carry the regret of it. This chapter exists so you won't.

But there is a second kind of unsaid — and that kind, I finished. I said everything to my father while he could still answer me back. I was not in the room when he died; that story belongs to Chapter 27. What I can tell you here is that the words I had already said are what let me survive it.

So don't save the practical conversations for the end. The last days are too late for the practical conversations — and never too late for the loving ones. If the last days are where you are right now, go to Chapter 28, and keep talking to them. For everything else, the right time is now, at a kitchen table, while the person you love can still answer you back.

Two conversations, then. The practical one — wishes, decisions, money, orders — which this chapter walks you through, question by question. And the love one, which only you can script.

Have both. Leave nothing unsaid.

CHAPTER 27

The Last Days Plan

My dad spent his last week at Baptist Hospital. The family gathered — my brothers and sister, my stepmom at his side.

I wasn't there. I want to tell you why.

That same week, my mom — who has Parkinson's disease — had made the trip to Raleigh to visit me. Watching her, I understood something nobody said out loud: this was probably the last trip she would ever be able to take. So I stayed. I did the math that every child of two declining parents eventually does. Dad would hold on a couple more days. I would honor Mom's last visit, then go to him.

Then, in the last days before Christmas, pneumonia set in. My wife and I sat together looking at flights. There were no good flights on Christmas. There almost never are. So we made the plan: we would drive down on Friday.

On Christmas morning, they started the morphine drip. He passed away that same morning.

We were going to drive on Friday. He didn't have a Friday.

My brothers had been calling. So had my stepmom. But what I heard in those calls was that she was held — the family was in town, and the support around my dad's bed was strong. My being there was not what that room was missing.

Here is the part I need to be honest about, because it is the reason this book has the title it has. At first, I did not feel guilty. I had made a real choice between two parents who needed me. I had said everything to my father long before that week — no forgiveness withheld, nothing I was still saving up to say. And the last memory I have of my dad is not tubes and machines. It is *him*.

The guilt came later, and it came from the outside. When the backlash from the family started, that is when *you should have been there* got into my head. That is when it set out to get me.

So I want to say something to whoever needs it: sometimes "it's your fault" is not even your own sentence. Someone hands it to you — grieving, angry, in the worst week of their lives — and you carry it as if it were yours. You are allowed to put it down. Grief turns loving people into scorekeepers. There was never a score.

And I owe you the rest of the truth. It is still painful to talk about what happened, to this day, and there are things I would have done differently. I am not writing to you from someplace healed. I am writing from inside the healing, and I will tell more of this story as I can.

I do this work for a living. I have stood in hospital hallways with a hundred families and told them the truth about last days: they cannot be scheduled. The timing belongs to no one. And when it was my own father, I still believed I had a couple more days. Nobody times it right. That is the point of this chapter.

A last-days plan is not about controlling death. Death does not take instructions. A plan is about deciding, before the last week, what matters most: who needs to be there, who calls whom, what he would want in the room, and what you will do when it happens faster than anyone believed. Not so the plan goes perfectly — so that whatever happens, you know you did what could be done. The rest was never yours to carry.

So when I say it's not your fault, understand what you're hearing. Not a slogan. Not a professional's comfort, handed down from somewhere above the pain. It is the sentence I am still learning to believe about myself — and I put it on the cover so we would both have to look at it.

CHAPTER 28

What to Expect in the Last Days

I have been in the room for the last days more times than I can count. And I can tell you what frightens families the most. It is not death. It is the not knowing.

Nobody tells families what dying looks like. So every change looks like an emergency. Every sound seems like suffering. Every hour, someone wants to call 911. The fear in the room doesn't come from what is happening — it comes from not knowing that what is happening is normal.

So let me tell you what I have watched at hundreds of bedsides, and what the hospice nurses and doctors taught me standing next to them — so that you can walk in prepared.

They will sleep more. Then more than that. Eating and drinking will fade — and this is where families struggle hardest, because food is how we love people. You will want to coax one more bite. Hear me: near the end, the body is finished with food. It is not starving. It is shutting down the systems it no longer needs, and forcing food and fluids doesn't add days — it adds discomfort. Offering is still allowed: sips and tastes for pleasure, mouth care for comfort. Offer. Never force.

The breathing will change. It may speed up, slow down, pause long enough to stop your own heart, then start again. Near the end it may rattle. I need you to know: it sounds far worse than it feels. That sound is harder on the people hearing it than the person breathing it. Comfort medications exist for exactly this, and the care team knows how to use them.

There may be restlessness. And there may be visions — reaching for things you cannot see, conversations with people who died years ago. I have stopped trying to explain this. I only know that it happens, that it is common, and that the dying are usually not frightened by it. More often, they seem comforted. You already know what to do, because it's the same thing I taught you in the dementia chapter: don't argue with their reality. Join it. But restlessness that

looks like distress is different — that one you report. The team can treat it.

Then there is the rally. One day — often near the very end — they may wake up clear. Alert, talking, maybe hungry, asking for people. Families take it as a turnaround. I have watched it break hearts twice for that reason. The rally is real, but it is usually not a recovery. It is a gift. Treat it like one: gather, say everything, laugh if there's laughing to do. It is often the last good day.

And through all of it — this most of all — **keep talking to them.** Hearing stays long after responding stops. No one can prove what reaches them, so we assume it all does. Say the things. None of it is wasted.

One more thing about the fear of the phone. When hospice is on board and death is expected, the number to call is the hospice line, not 911. It answers twenty-four hours a day, and it exists for exactly these hours — for any change, and for any fear, at 2 a.m. as much as 2 p.m.

Knowing all of this will not make the last days easy. But it changes what you carry into the room. Instead of fear, you can bring presence. That is what this chapter is for.

CHAPTER 29

After Death

My dad died on Christmas morning.

And then came the part nobody prepares families for: after.

Here is what my family had. My dad had told my stepmom his wishes — he wanted to be buried. The *what* was known. Here is what no one had: the *how*. There was no financial plan. No conversation, ever, about what a burial costs or where that money would come from.

So the money questions came due during funeral week — the one week that exists for grieving — and they pulled us away from the only thing that week was for.

I told you in Chapter 5 about the meeting my family never had. I'm telling you now what it cost. There are two plans, and most families have at most one of them. The wishes plan: what they want, where, how. The financial plan: what it costs and where the money comes from. My family had the first. Nobody had the second. Grief and a checkbook do not belong in the same week — and the only way to keep them apart is to make both plans while everyone is still alive.

The rest of this chapter walks you through those first days — the calls, the funeral home, the practical tasks that arrive whether you are ready or not — and the weeks after, which nobody talks about at all. Take everything slowly. There is no stopwatch on any of it.

But I want to end with what happened fifteen days later.

On the morning of January 9th — seventeen years, to the day, since I got sober — a cardinal landed on the branch outside my window.

I'm not going to explain it. I told you in the last chapter that I've stopped trying to explain things like this. I'll only tell you what happened: I stood at the window and I watched it, and I did not feel alone.

That is the last thing this book has to say to you. The love does not end on the day they die. It keeps arriving — in the tasks you finish for them, in the words you said while they could still hear you, in whatever lands on the branch outside your window when you need it most.

You showed up. You are still showing up.

It's not your fault. It never was.

OPENERS STILL TO COME

Chapter 4 — Caregiver Burnout

Chapter 6 — Home Health

Chapter 7 — Private Duty Care

Chapter 8 — Assisted Living

Chapter 11 — Palliative Care

Chapter 13 — Medicare Basics

Chapter 14 — Medicaid Basics

Chapter 15 — Private Pay

Chapter 16 — Long-Term Care Insurance

Chapter 18 — VA Benefits

Chapter 23 — Medical Orders: DNR, MOST, and POLST

Chapter 25 — Medication and Doctor Lists

Chapter 26 — Emergency Contacts and Crisis Plan